

Kerala Expansion Playbook

For Chairman review

EXECUTIVE READ Kerala is in a 24-month consolidation window. Aster–Quality Care merger (Nov 2024) + KKR's Baby Memorial roll-up have locked the premium top. VPS is structurally sub-scale in a consolidating market. Our edge — operating depth, Gulf-NRI catchment, brand-neutrality for mission hospitals, and operating-distress execution — gives us a defensible right to win in four channels: mid-market family acquisitions, mission O&M, NCLT/IBC distressed resolution, and greenfield only where no brownfield exists. Decision asks: (i) capital envelope ₹2,000–2,500 Cr over 24 months; (ii) approve a Head of Corp Dev hire; (iii) green-light bilateral outreach on 6 priority Tier-1 names and the SUT Royal NCLT track.

What we recommend

Channel	Capacity	Top targets	Capital	Action this quarter
Mid-market family acquisitions	2 deals / 18 mo	Renai, Sunrise, Indo-American, Mother	₹800–1,100 Cr	NDAs on 4 names
Mission O&M concessions	2 concessions / 18 mo	Lourdes, Caritas, Bishop Benziger, Pushpagiri	₹400–500 Cr capex	Cardinal/Synod-level engagement
NCLT / IBC distressed	1–2 resolutions / 18 mo	SUT Royal (TVM), PVS Memorial (Kochi)	₹100–250 Cr	Empanel counsel, file EoI for SUT
Greenfield (only if needed)	1 site / 18 mo	TVM Technopark, Kollam NH-66, Kakkanad IT	₹450–600 Cr	Site option only — defer build

Total ask: ₹2,000–2,500 Cr deployable envelope · 24-month execution · 4–6 deals closed. Hospital-by-hospital strategic detail in the companion 17-hospital deep-dive deck.

1. Strategy

Why expand · why now · why VPS

Why expand. Lakeshore Maradu has the quaternary capability (oncology, transplant, complex cardiac, neuro). Without spokes, we capture only Ernakulam. With spokes, specialist utilisation multiplies 2–3×, ARPOB lifts via payer-mix shift, and quaternary referrals stabilise hub P&L through cycle.

Why now. Aster–QCIL merger (Nov 2024) created the structural market leader: 38 hospitals nationally, ~3,500 Kerala beds + 600 under construction, ₹850 Cr 3-year Kerala capex. KKR has done 3 Kerala deals in 24 months. By FY28 the premium independent asset pool will be exhausted. Our window is 18–24 months.

Why VPS. PE has more capital than us. PE does not have multi-hospital UAE operating experience, the Gulf-NRI catchment, or brand neutrality for mission-hospital partnerships. That is the gap we sell — not capital.

Our posture — four channels, not one

- Mid-market family acquisitions where succession dynamics open the seller
- Mission/trust O&M concessions where capital + operating capability bridges the gap that mission governance cannot deploy alone
- NCLT/IBC distressed resolution where speed + Sec 29A readiness wins at 50–70% discount
- Greenfield only where no brownfield exists and the catchment justifies the build

Should we look at loss-making targets?

Yes — but only when the loss is operating, not structural. The 2×2 below governs the decision.

	Attractiveness STRONG	Attractiveness WEAK
Currently profitable	O&M / JV. Owner won't sell cheap → extract upside via service-line CoE or management partnership (Caritas, Bishop Benziger, Mother).	Pass. No urgency on either side.
Currently loss-making	BUY — turnaround play. Distress + good bones = 40–60% valuation discount + 18-month operating fix (Renai-type).	AVOID. Structural decline destroys value — working-capital drain, brand contagion, talent flight.

RULE OF THUMB If loss is <5% of revenue and structural drivers (location, demographic, clinical reputation, insurance empanelment) are intact → buyable at 50–60% of profitable-peer multiple. If loss >15% of revenue OR structural drivers broken → walk. The 5–15% zone needs forensic clinical + commercial DD.

Loss-making — trigger signals

Investigate	Walk away
Founder-doctor succession failure (revenue declining despite stable infrastructure)	Wrong-location asset (built in a bypassed corridor or declining demographic pocket)
Over-capex of 2018–22 cycle compressing margins; underlying clinical ops fine	Doctor exodus already complete; rebuilding takes 5–7 years and fails often
Empanelment crisis (lost MEDISEP/insurer due to billing issues); 12-month turnaround	Quality / safety scandal in local media — brand cannot recover under same identity
Family / governance dispute paralysing investment decisions	Title / land disputes; insurance or NABH blacklisting

2. Where we play

Kerala's fourteen districts behave very differently. The matrix below tells us where to lean in, where to watch, and where not to spend Chairman time.

District	Catchment / NRI	Competitive structure	VPS posture	Priority
Ernakulam	Premium urban + IT belt	Aster Medcity + Amrita; we operate Maradu	Defend share + add spokes	Priority 1
Thiruvananthapuram	Tier-1 + IT corridor	KIMS dominant; Aster Capital opens 2026	Enter via SUT NCLT or Cosmopolitan/GG	Priority 1
Kottayam	Central Travancore Knanaya NRI	Only KIMS at sub-scale	Mission O&M (Caritas) — geographic lock	Priority 1
Pathanamthitta	Highest Gulf-NRI density	Believers, Pushpagiri, TMM — all mission	JV + CoE (Pushpagiri, BCMCH)	Priority 1
Thrissur	Largest district, no PE anchor	Amala mission only at scale	Equity (Mother) + greenfield optional	Priority 1
Kollam	Underserved south Kerala	Only KIMS at scale	Mission O&M (Bishop Benziger) + greenfield	Priority 2
Alappuzha	Backwater catchment	Limited tertiary	Specialty bolt-on (Indo-American)	Priority 2
Idukki / Wayanad	Hill catchment	Sparse private capacity	Affiliate spoke only	Priority 3
Kozhikode / Malappuram / Kannur / Kasaragod	North Kerala	KKR-BMH + Aster MIMS locked	Do not enter competitively	Avoid
Palakkad	Tamil Nadu border	Sparse	Monitor (Karuna optionality)	Priority 3

IMPLICATION Concentrate Chairman time on the five Priority-1 districts (Ernakulam, Thiruvananthapuram, Kottayam, Pathanamthitta, Thrissur). They host 9 of the 11 Tier-1 names and 95% of the expected value creation. North Kerala is structurally locked — do not contest.

3. Playbook — deal structures

Seven structures across the equity-to-services spectrum. Structure is chosen by the seller's risk preference, our capital appetite, and the asset's role in the network.

#	Structure	Use for	Capital	Control	Timeline
01	Full equity acquisition (60–100%)	Family-run mid-size with succession trigger	High	Full	6–9 mo
02	Long-tenor O&M concession (15–25 yr)	Mission / trust hospitals, religious institutions	Low	Operational	9–12 mo
03	Management contract (5–10 yr)	Distressed assets, owner reluctant to exit	Minimal	Operational	4–6 mo
04	JV (50:50 or 51:49) NewCo	Medical colleges, large institutional partners	Moderate	Shared	9–14 mo
05	White-labelled CoE carve-out	High-volume hospitals lacking a specific capability	Low-Mod	Service line	4–7 mo
06	Clinical affiliate network	District-level spokes, light-touch entry	None	Clinical only	2–3 mo
07	NCLT Resolution Plan	Hospitals in CIRP at NCLT; distressed assets	Low-Mod	Full post-approval	9–15 mo

Selection framework — non-negotiables

Every target — regardless of headline score — fails if any of these is present:

- Contested or unclear real-estate title
- Criminal / PMLA proceedings against promoters
- Active regulatory suspension or major NABH withdrawal
- Doctor attrition >25% over trailing 24 months
- Bed occupancy below 50% with no recovery trajectory
- Litigation / contingent liabilities >30% of asset value
- Active labour dispute with KGMOA / INA / KGOA unresolved

Five-phase execution sequence

Phase	Months	Key deliverables
1 Strategic mandate	0–1	Capital envelope · Corp Dev cell stood up · advisor panel · screening criteria
2 Pipeline build	1–3	Long-list 60+ → score → filter to 20 → Chairman heat-map
3 Bilateral outreach	3–4	8–10 NDAs · indicative term sheets · site visits · shortlist 4–5

4 Diligence & structuring	4–7	Commercial / clinical / financial / legal / title DD · definitive agreements
5 Integration	7–24	Day-1 playbook · brand transition · clinical SOP rollout · talent migration · CoE build

4. NCLT / IBC channel

Kerala has two named hospitals in active NCLT distress today. This is a legitimate parallel acquisition channel — distinct economics from conventional M&A, materially under-fished, and about to become more active with the IBC Amendment Act 2026 (creditor-initiated insolvency).

Two named live cases

SUT ROYAL HOSPITAL, TRIVANDRUM — UNDER LIQUIDATION ~400 beds. Corporate debtor TIHSL under NCLT-ordered liquidation after Sabine Hospital failed as resolution applicant. Royal-patronage brand with cardiac depth. Liquidation EV est. ₹100–180 Cr vs open-market equivalent ₹400+ Cr (55–70% discount). Cleanest Trivandrum entry without paying Aster/KIMS multiples and beats Aster Capital TVM (454 beds, 2026 opening) to market.

PVS MEMORIAL HOSPITAL, KOCHI — CIRP, STATUS UNCERTAIN ~200 beds. CIRP admitted Oct 2019 (operational creditors); withdrawal applications + promoter appeals subsequently filed. Status complicated. Kaloor location mirrors Renai's urban Kochi second-asset logic. Best approach: bilateral with promoter outside CIRP, with NCLT as fallback.

Process — 4 stages, 9–15 months

Stage	Window	What VPS does
1 EoI & qualification	14–30 days post Form A	Sec 29A clearance, capital envelope pre-cleared, submit EoI
2 RA shortlisting + diligence	~30 days	Information memo + data room; site visit; consultant gap analysis
3 Plan submission + CoC vote	~30 days	Resolution Plan + 5–10% PBG; CoC 66% vote
4 NCLT sanction + takeover	~3–6 months	Asset transferred clean of pre-CIRP claims (Essar Steel precedent)

NCLT vs conventional M&A

Dimension	NCLT	Conventional M&A
Price	Significantly discounted (haircut on debt)	Market multiple + control premium
Legacy liabilities	Extinguished on plan approval	Survive — must be indemnified
Title	Clean of pre-CIRP claims	Carries historical encumbrances
Doctor / staff	Often departed during CIRP — bench rebuild needed	Founder buy-in retains team
Competition	Open auction — any Sec 29A bidder	Bilateral or controlled auction
Brand	Can rebrand immediately	Constrained by founder sentiment

What VPS needs to be ready

- Pre-cleared capital envelope (₹250 Cr per plan) — CIRP timelines do not accommodate corporate approval cycles
- Empanelled NCLT specialist counsel (Cyril Amarchand / Trilegal / Cochin-based firm) on retainer
- Sec 29A clearance for all VPS group entities (Lakeshore, VPS Healthcare UAE, related shareholders)
- Performance bank guarantee line (₹25 Cr) pre-arranged with Federal Bank / SBI Kerala

- Going-concern operating SOP — 100-day Day-1 integration template for distressed hospitals
- Weekly cause-list review of NCLT Kochi Bench and IBBI Form A announcements

5. Targets

Eleven Tier-1 names engaged actively in next 90 days. Full hospital-by-hospital deep-dives (profile + investment case + roadmap) in the companion deck.

Tier-1 deep dives — the 11 priority names

#	Target · location	Beds	Structure	EV (₹ Cr)	Why it matters
1	Renai Medicity · Kaloar, Kochi	250	Equity 60–70%	250–350	Defensive Kochi #2; pre-empts Aster/KKR; distress signals
2	Sunrise Hospital · Kakkanad	200	Equity 70% + 30% rollover	200–280	W&C/IVF gap; Kerala's highest-PPP catchment; pure additive
3	Lourdes Hospital · Pachalam, Kochi	600	25-yr O&M concession	150–200 capex	Largest Kochi opportunity; combined network = #1 Kochi private
4	Caritas Hospital · Kottayam	700	25-yr managed services	CoE capex	Central Travancore lock; Knanaya Gulf-NRI pipeline
5	Pushpagiri Medical College · Tiruvalla	700	JV 51:49 / 50:50	JV contribution	Network HoD pipeline; medical-college regulatory moat
6	Believers Church MC · Tiruvalla	743	CoE 7–10 yr	50–80 capex	Newest plant; lowest integration risk; oncology entry
7	Indo-American · Vaikom border	200	Full equity 75–100%	150–220	Cleanest pure-fit; Day-30 tumour-board integration
8	Bishop Benziger · Kollam	300	20-yr O&M concession	75–100 capex	Best non-PE Kollam asset; NH-66 southern hub
9	SUT Royal · Trivandrum (NCLT)	400	NCLT resolution	100–180	55–70% discount; cleanest TVM entry pre-Aster Capital
10	Cosmopolitan / GG · Trivandrum	300/350	Equity 60–80%	250–400 per asset	Backup to SUT; both family + succession decade
11	Mother Hospital · Thrissur	350	Full equity 80–100%	300–450	Largest un-consolidated district; #1 from Day-1

Tier 2/3 active monitoring + indicative + competitor benchmark

Target	District	Beds	Posture	Note
Welcare Hospital	Ernakulam	120	T3	Suburban Kochi spoke; defends Maradu south flank
Daya General Hospital	Thrissur	250	T2	Fallback to Mother Hospital if primary track stalls
Moulana Hospital	Malappuram	600+	Indicative	Outside formal table; defensive partnership only (NK locked)
Meditrina Hospital	Kollam	200	T3	Cardiac-specialty clinical affiliate; pairs with Bishop Benziger
Almas Hospital	Kottakkal	200	T3	Malappuram NRI affiliate; pairs with Moulana indicative

Aster Medcity	Kochi	690+10 0	Competitor	Profiled as benchmark — NOT a target
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Full target universe — 52 candidates summary

Detailed 52-candidate table in companion deep-dive deck. Distribution: Ernakulam 13 · Pathanamthitta 6 · Thrissur 6 · Thiruvananthapuram 6 · Kollam 5 · Kottayam 5 · Malappuram 4 · Alappuzha 3 · Kozhikode 3 · Kannur 2 · Wayanad 1 · Idukki 1 · Kasaragod 1 · Palakkad 1. Two named NCLT cases (SUT, PVS Memorial).

6. Greenfield zones

Ten zones across Kerala. Three are priority for site-option lock-in in FY27; the rest are monitoring. Brownfield is preferred where available — greenfield is the fallback.

Zone	Catchment	Beds	Capex (₹ Cr)	Priority	Note
Trivandrum — Technopark / Kazhakkootam	~12L IT	300–400	450–600	Priority 1	Highest-PCI corridor; pair with brownfield (Cosmopolitan/GG)
Kollam — NH-66 bypass (Polayathode / Kundara)	~26L district	200–300	300–400	Priority 1	Underserved between TVM and Kochi; redundant if Bishop Benziger O&M closes
Kochi — Kakkanad Infopark satellite	~5L IT belt	100–150	150–220	Priority 1	Specialty day-care satellite feeding Maradu; pair with Sunrise
Thrissur — NH-544 (Mannuthy / Kuriachira)	~32L district	250–350	350–500	Priority 2	Alternative if Mother Hospital deal does not progress
Tiruvalla — Pathanamthitta NRI belt	~12L NRI	150–250	250–350	Priority 2	Mission-incumbent dense; brownfield (BCMCH CoE / Pushpagiri JV) preferred
Kannur — international airport corridor	~25L + NRI	200–300	300–400	Priority 3	Long ramp; CPI-M political sensitivities
Wayanad — Sultan Bathery wellness + tertiary	~8L + visitors	100–150	150–220	Priority 3	Hill catchment + wellness segment; small district
Mavelikara / Chengannur — mid-Travancore gap	~10L	100–150	150–220	Priority 3	Marginal standalone; only as hub-spoke with Lakeshore / Bishop Benziger
Kochi — Vyttila / Edapally specialty CoE	Citywide	80–120	120–180	Priority 3	Standalone Cancer / Heart specialty CoE; volume risk
Vizhinjam — port-city long-term play	Future port pop.	150–200	Land option only	Watch	10-year demographic play; trigger at port-Phase 2 commissioning

GREENFIELD PRINCIPLE Build only where (i) no acceptable brownfield exists in target geography, (ii) the catchment justifies the build economics standalone, and (iii) the site adds defensive value to the network. Three sites should be locked in FY27 as land-option only — capital deployment decision sequenced against brownfield outcomes.

7. Next steps — first 90 days

Three parallel streams — institutional, pipeline, capability. Chairman review at Day 30, 60, 90.

Stream 1 — Institutional (mandate & governance)

- Board mandate paper — capital envelope ₹2,000–2,500 Cr, NCLT route up to ₹250 Cr per plan. Chairman approves by Day 30.
- Corp Dev cell — Head of Corp Dev hire + CFO secondee + clinical advisor + M&A counsel. Live by Day 45.
- Advisor panel — IB (deal flow), Big 4 (FDD/CDD), top-tier law firm, NCLT specialist. RFPs out by Day 21.
- Sec 29A clearance for VPS group entities. Audit by Day 45.
- Performance bank guarantee line with Federal Bank / SBI Kerala. Day 60.

Stream 2 — Pipeline (priority bilateral engagement + NCLT track)

- Renai Medicity — banker-channelled approach; NDA + indicative offer in 60 days
- Sunrise Hospital — direct founder-to-Chairman channel; first meeting in 30 days
- SUT Royal (NCLT) — engage insolvency counsel immediately; submit qualifying interest within 30 days; PBG arranged
- Bishop Benziger (CSI / Kollam) — Diocese leadership engagement; Chairman-led discovery
- Indo-American Vaikom — founder-doctor conversation via tumour-board relationships; NDA by Day 60
- Caritas (Knanaya) — long-fuse community engagement; no deal proposal yet
- NCLT Kochi monitoring — weekly cause-list + IBBI Form A scan; standing weekly memo

Stream 3 — Capability (the offering we sell)

- O&M offering deck — branded sales document for mission/trust hospitals. Day 45.
- UAE network reference site visits for 2–3 priority trust-hospital boards. Day 60–90.
- Hub-spoke operating model — tumour board, telemedicine, locum rotations protocol. Day 90.
- Day-1 integration playbook (brand transition, clinical SOP, HR migration, IT/EMR) — living document.
- Talent map — internal CEO/CMO/HoD candidates for 5 spoke deployments. Day 90.

THE SINGLE MOST IMPORTANT FIRST MOVE Hire a Head of Corporate Development. This is currently a vacuum in the Lakeshore executive team. The right hire (ex-banker + ex-hospital operator) accelerates everything else by 6–9 months. Approve the role brief in the next leadership review.